



MODULE 4 — MEDICAL TERMINOLOGY & DOCUMENTATION

AfyaDesk Remote Medical Careers Course | Speak the Language, Respect the Boundary

Estimated study time: 8–10 hours (reading + 200-term deck + 4 documentation labs)

Prerequisites: Modules 1–3 (roles + reliability + MVA workflows)

What you will produce: 200-term personal glossary + 4 formatted documents (SOAP admin note, referral letter, discharge packet, plain-English rewrites) + dangerous-abbreviation audit.

Learning Objectives

By the end of this module, you will be able to:

1. Decode 200+ prefixes, roots, suffixes, and abbreviations to read orders, referrals, and inboxes without freezing.
2. Use anatomy, disease, medication, lab, and imaging terms accurately in scheduling, filing, and summaries.
3. Distinguish safe vs Joint Commission Do-Not-Use abbreviations and apply patient-safe writing rules.
4. Write factual, timestamped SOAP-aligned admin notes that auditors trust.
5. Assemble referral letters, discharge packets, and routine correspondence that prevent rejections and callbacks.
6. Translate jargon into plain English for patients with low health literacy and cross-cultural needs.
7. Enforce admin vs clinical decision boundaries with scope-safe scripts and escalation documentation.
8. Proofread with a 3-pass system to achieve >98% accuracy and <24-hour turnaround.

1. Why Terminology Equals Employability (And Patient Safety)

Imagine this inbox at 4pm EAT: “Need prior auth for MRI L-spine ? disc herniation, pt with sciatica radiating L leg, on NSAIDs, PMH HTN, allergy PCN rash — STAT, neuro deficit ? foot drop.” If





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you freeze, misfile under “general X-ray,” or transcribe “sciatica” as “psoriasis,” you delay neurosurgery triage by days.

Second scenario: doctor dictates “Metoprolol 50 mg BID, hold if SBP <100 or HR <60.” You type “Metoprolol 500 mg BID” — tenfold overdose on permanent record. Third: you tell portal patient “SOB is normal after surgery” misunderstanding SOB as “sobbing” not shortness of breath — you reassured a pulmonary embolism.

You will never diagnose, but you must spell, file, schedule, summarize, and read back correctly. Terminology is not vocabulary test — it is triage accuracy, scheduling safety, and documentation liability. Employers pay \$2–\$4/hr premium for VAs who “speak medicine” because they need fewer corrections and cause fewer near-misses.

Kenyan advantage: you already use these terms on wards — HTN, DM, CCF, PUD, UTI. Gap: US spelling and usage (esophagus not oesophagus, acetaminophen not paracetamol, ER not casualty, PCP not MO) plus insurance-linked language (ICD-10, CPT, prior auth medical necessity). This module bridges both.

Study system for this module: 20 new cards/day in Anki/Quizlet (term → breakdown → plain English → example sentence), say aloud 10 min/day (pronunciation matters on calls: “atenolol” vs “atorvastatin” confused by phone calls), use each term in a scheduling sentence (“Patient with otitis media booked to ENT, 30-min new, insurance verified”). Test 100 terms mid-week, 200 end-week, aim 90%+. Keep deck for interview prep — employers quiz 10 terms live.

2. Building Blocks: Prefixes, Roots, Suffixes — Decode Any Word in 10 Seconds

Master 60 building blocks and you can decode 70% of unfamiliar terms without Google.

2.1 Prefixes (before root — number, position, time, negation)

a-/an- (no, not: afebrile, anuria), anti- (against: antihypertensive), brady- (slow: bradycardia <60), tachy- (fast: tachycardia >100), hyper- (high: hypertension), hypo- (low: hypoglycemia), dys- (difficult/painful: dyspnea, dysuria), eu- (normal: euthyroid), mal- (bad: malnutrition), neo- (new: neonatal), pre-/post- (before/after: preoperative), peri- (around: perinatal), intra-/inter- (within/between: intravenous, intercostal), sub- (below: subcutaneous), supra- (above: supraclavicular), hemi- (half: hemiplegia), poly- (many: polypharmacy), oligo- (few: oliguria), aniso- (unequal).



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Practice: hypothyroid = hypo + thyroid (low thyroid function → fatigue, cold intolerance → schedule routine endocrine, not urgent unless myxedema red flags). Hyperkalemia = hyper + kal (potassium) + emia (blood) → high potassium → urgent labs + ECG per SOP + escalate, never advise diet alone.

2.2 Roots (body system core)

cardi (heart: cardiology, myocarditis), pulmon/pneumo (lung: pulmonary embolism, pneumonia), nephro/ren (kidney: nephrology, renal failure), hepat (liver: hepatitis), dermat (skin: dermatitis), osteo (bone: osteoarthritis), arthr (joint: arthritis), myo (muscle: myalgia), neuro (nerve: neuropathy), encephal (brain: encephalopathy), gastro (stomach: gastritis), enter (intestines: gastroenteritis), col (colon: colonoscopy), cyst (bladder: cystitis), hyster (uterus: hysterectomy), oophor (ovary), orchid (testis), mamm (breast: mammogram), ophthalm (eye), ot (ear: otitis media), rhin (nose: rhinitis), laryng/pharyng (throat), hem/hemat (blood: hematology, anemia), leuk (white: leukocytosis), thromb (clot: thrombosis), angi/vas (vessel: angiography), lymph (lymph: lymphedema), endocrin (hormone).

Combine: gastro + enter + itis = stomach + intestines inflammation → schedule same-day if dehydrated child/elderly, routine if mild adult with fluids tolerated. Nephrectomy = nephro + ectomy (removal) → urology/nephrology follow-up, not cardiology — filing matters.

2.3 Suffixes (procedure, condition, disease)

-itis (inflammation: appendicitis → urgent surgical), -ectomy (removal: appendectomy), -otomy (cut into: tracheotomy), -ostomy (new opening: colostomy), -plasty (repair: angioplasty), -scopy (look inside: colonoscopy), -graphy (imaging: mammography), -gram (image: ECG), -algia/-dynia (pain: myalgia), -emia (blood: anemia), -uria (urine: hematuria), -pathy (disease: neuropathy), -osis (abnormal: osteoporosis), -oma (tumour: carcinoma vs benign adenoma — never assume benign/malignant, use exact term), -penia (low: thrombocytopenia).

Drill: pericarditis (peri + cardi + itis = around-heart inflammation → chest pain → urgent cardiology/ER track, not routine). Hyperglycemia (high blood sugar) vs hypoglycemia (low — shakiness, confusion → urgent same-day advice to seek care + escalate, never “eat sugar and wait” as VA diagnosis).

Keep a 3-column sheet: Term | Breakdown | What VA does (schedule where, which urgency, what to verify). Example: dysuria (dys + uria = painful urination → UTI likely → same-day/telehealth, verify fever, allergy, pregnancy for scheduling notes, no antibiotic advice). This sheet becomes interview proof.





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3. Anatomy and System Terms for Correct Scheduling and Filing

You schedule by system + urgency, not just doctor name. Wrong specialty wastes 3 weeks and triggers 1-star review.

Cardiovascular: hypertension (HTN), hypotension, myocardial infarction (MI/heart attack), angina, arrhythmia, atrial fibrillation (AFib), congestive heart failure (CHF — SOB, edema, weight gain), hyperlipidemia, deep vein thrombosis (DVT — leg swelling/pain → urgent Doppler), pulmonary embolism (PE — SOB + chest pain → ER now). Admin: chest pain, pressure, fainting, palpitations with dizziness → ER/urgent cardiology track, never routine. Verify BP log, ECG, echo if ordered.

Respiratory: asthma, COPD, pneumonia, bronchitis, tuberculosis (TB — Kenyan context, airborne precautions note for in-person), shortness of breath (SOB), wheeze, hemoptysis (coughing blood → urgent). Schedule wheeze + fever child same-day, routine inhaler refill via portal if stable.

Endocrine: diabetes mellitus Type 1 (insulin-dependent) / Type 2, HbA1c (3-month sugar average — target <7% most adults), hypo/hyperglycemia, hypothyroid (fatigue, cold) / hyperthyroid, Addison/Cushing (rare — file to endo). Admin: diabetic foot wound + fever → same-day, routine HbA1c review 20-min.

Musculoskeletal: fracture (fx), sprain/strain, osteoarthritis, rheumatoid arthritis, gout, sciatica (leg pain from L-spine nerve → physio/spine, verify MRI auth), low back pain (LBP), osteoporosis. Verify laterality every time: “Left or right knee? Since when? Injury or gradual?” Wrong knee MRI is never-event.

Neurology: cerebrovascular accident (CVA/stroke — FAST: face droop, arm weakness, speech change, time → ER immediately), transient ischemic attack (TIA — warning stroke → urgent same-day), migraine, tension headache, seizure/epilepsy, Bell’s palsy, neuropathy (numbness). Headache with “worst ever,” vision change, weakness → ER track + escalate, never routine.

Gastrointestinal/Genitourinary: GERD (reflux), peptic ulcer (PUD), gastritis, appendicitis (RLQ pain + fever → ER), cholecystitis, hepatitis, cirrhosis, urinary tract infection (UTI — dysuria, frequency → same-day, verify pregnancy/allergy), kidney stones (flank pain → urgent), chronic





kidney disease (CKD — creatinine/eGFR tracking), benign prostatic hyperplasia (BPH). Admin: vomiting blood, black stools, no urine 8 hrs → urgent.

OB/GYN/Peds/Geriatrics basics for scheduling: antenatal (ANC), postpartum, Pap smear (cervical screening), mammogram, contraception refill (routine), well-child vaccines, developmental milestones, falls in elderly + polypharmacy review (book longer). Always note pregnancy/breastfeeding for medication-related scheduling flags — you flag, clinician decides.

Filing rule: file by problem + specialty, not symptom alone. “Chest pain + HTN + abnormal ECG” → cardiology, not “general.” “Hematuria + flank pain” → urology, not GP routine. When unsure, ask clinician with SBAR, don’t guess specialty.

4. Abbreviations: Lifesavers, Time-Savers, and Career Killers

4.1 Safe common (know, but expand for patients)

Vitals/visits: BP, HR, RR, Temp, SpO₂, BMI, CC (chief complaint), HPI, ROS (review of systems), PMH (past medical history), PSH (past surgical), FH (family), SH (social), ALL (allergies — always verify), MEDS, Dx (diagnosis), Tx (treatment), Rx (prescription), Fx (fracture/follow-up context), FU (follow-up), STAT (immediately), PRN (as needed), NPO (nothing by mouth), N/V/D (nausea/vomiting/diarrhea), SOB (shortness of breath), CP (chest pain), HA (headache), UTI, URI, LBP, ROM (range of motion).

Use in team notes/tasks, never in patient SMS/email without expansion: “Take 1 tablet twice daily with food” not “1 tab BID PO.” Elderly and non-native speakers misread abbreviations dangerously.

4.2 Joint Commission Do-Not-Use (memorize — audit fails you for these)

U for unit (write unit — U mistaken as 0 → 10U insulin read as 100 → fatal), IU for International Unit (write International Unit), QD/QOD for daily/every other day (write daily), trailing zero 5.0 mg (write 5 mg — 5.0 read as 50), lack of leading zero .5 mg (write 0.5 mg), MSO₄/MgSO₄ (write magnesium sulfate / morphine sulfate fully), @ (write at). As VA, never create these in transcriptions — correct to safe form and flag original to clinician.

Sound-alikes to verify by read-back: Celebrex/Celexa/Cerebyx, Metformin/Metronidazole, Amlodipine/Amitriptyline, Prednisone/Prednisolone, Hydroxyzine/Hydralazine, fifteen/fifty, hyper/hypo, left/right, QD/QID. Repeat + spell: “Metformin M-E-T-F-O-R-M-I-N 500 mg twice daily — correct?”





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4.3 US vs UK vs Kenya usage

Paracetamol (UK/Kenya) = acetaminophen/Tylenol (US), A&E (UK) = ER/ED (US) = Casualty (older Kenya), GP (UK/Aus) = PCP/family doctor (US) = MO/CO (Kenya), theatre (UK) = OR (US), chemist (UK) = pharmacy (US), notes (UK) = chart (US). Dosing: mg vs mcg — 1000x difference, verify decimal + unit twice for peds. Dates: US 05/06 = May 6, UK 05/06 = 5 June — always write “06 May 2026.”

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5. Diseases, Medications, Labs, and Imaging Vocabulary for Admin Accuracy

5.1 Medication families (recognize, spell, never advise)

Antihypertensives: ACE inhibitors (-pril: lisinopril, enalapril — cough side effect note for triage), ARBs (-sartan: losartan), beta-blockers (-olol: metoprolol, atenolol — hold if HR <60 per order only), calcium-channel blockers (-dipine: amlodipine), diuretics (HCTZ, furosemide — urination, potassium). Diabetes: metformin, glipizide, insulin (glargine, lispro — units, never U), Jardiance/Ozempic (new — verify spelling). Statins (-statin: atorvastatin — muscle pain flag), blood thinners (warfarin, apixaban/Eliquis — bleeding risk → urgent if bleed + on thinner), NSAIDs (ibuprofen, diclofenac — ulcer/kidney), antibiotics (-cillin: amoxicillin, -mycin: azithromycin — allergy critical), inhalers (albuterol rescue vs steroid controller), antidepressants (sertraline, escitalopram), thyroid (levothyroxine — morning empty stomach).

Admin rules: verify name + dose + frequency + route + quantity + refills via read-back, check allergy list before documenting refill request, flag interactions to clinician (don't counsel), never refill without order, document “refill request sent to Dr. X pool 2pm ET, patient advised expected reply 24h, worsening → urgent care.”

5.2 Labs and vitals you will chase and file

CBC (anemia/infection), BMP/CMP (kidney/liver/electrolytes — creatinine/eGFR for CKD staging), HbA1c (diabetes control), lipids (cholesterol), TSH (thyroid), urinalysis (UTI), urine culture, Pap smear (cervical), PSA (prostate — verify consent discussion done), STI panel, TB GeneXpert



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(Kenya), malaria RDT, HIV viral load/CD4 (sensitive — extra privacy). Vitals: BP (normal <120/80, HTN $\geq$ 130/80 US), HR 60–100, SpO₂ >94%, Temp, weight (dose, HF monitoring — 2kg gain in 3 days → flag).

When lab result arrives via portal: do NOT interpret to patient (“Your creatinine is a bit high but okay” forbidden). Script: “Your results are in — Dr. X will review and message you by [time]. For worsening symptoms meanwhile, please seek urgent care. Can I help book follow-up?” Route to clinician pool STAT if flagged critical (lab calls critical — phone immediately per SOP).

5.3 Imaging and procedures language

X-ray (fracture, chest), ultrasound (US — pregnancy, gallbladder, DVT Doppler), CT (trauma, stones — contrast allergy check), MRI (L-spine disc, knee — claustrophobia/metal screening + prior auth common), ECG/EKG (heart tracing — chest pain → STAT), echocardiogram (heart function), mammogram (breast screening — recall if dense), Pap/colposcopy, endoscopy/colonoscopy (prep instructions — NPO, laxative — confirm understanding via teach-back), biopsy (pathology pending → track 7–14 days, never guess benign).

Auth tip: MRI/CT often need prior auth with notes + failed conservative therapy + conservative duration. Your packet completeness decides approval in 3 vs 14 days. Checklist: order + indication + ICD + prior imaging + labs + clinic notes + auth form + patient contact.

6. Clinical Documentation You Will Touch Daily (And How Not to Create Liability)

6.1 SOAP structure and where VAs fit

Subjective (patient says: “Headache 3 days, 7/10, with blurred vision”), Objective (vitals, exam, labs: BP 150/95, HbA1c 8.2%), Assessment (clinician diagnosis: “Migraine vs hypertensive urgency”), Plan (labs, meds, referrals, follow-up). VA writes S verbatim + accurately, files O from devices/forms without interpreting, never writes A/P. Example admin note that is safe:

14 May 2026 16:10 EAT / 09:10 ET – Inbound call from Smith, J DOB 1980-05-14. Pt reports headache 3 days with blurred vision today. Denies chest pain per triage Qs. Advised urgent same-day evaluation per SOP, offered 11:20am ET today + ER precautions given. Accepted 11:20am. SMS + email confirmation sent with Dual-zone. Flagged to Dr. Lee pool as urgent 09:15 ET. – Mercy N, MVA

Unsafe version (never write): “Pt has migraine, reassured, told to take Panadol.” That is diagnosis + advice + liability.





6.2 Referral letters that get accepted first time

Header (practice letterhead, date both zones), To (specialist name, fax/portal, phone), Re (patient full name, DOB, MRN, insurance + auth), Reason + urgency (routine <14d / urgent <72h / STAT), History relevant (PMH, meds with doses, allergies bolded, social if relevant), Findings (vitals, labs with dates, imaging attached list), Question asked ("Please evaluate LBP with foot drop for ? disc herniation — MRI attached"), Contact (PCP phone + VA callback + patient phone + consent to contact). Attach: notes, labs, imaging discs/links, auth. Number pages. Confirm receipt in 24h. Rejection audit: 90% missing clinicals or illegible — attach before sending, call to confirm readable.

6.3 Discharge packets and correspondence

Discharge: summary (admission reason, procedures, condition at discharge), med list reconciled (new vs stopped vs continued with doses/times), follow-up appt booked before discharge (date/zone/link), red-flag instructions ("Return for chest pain, bleeding, fever >101°F, wound redness spreading"), who to call (day/night numbers), wound/diet/activity limits in plain language. Teach-back: "To be sure I explained clearly — when is your follow-up and what 2 signs bring you back now?"

Routine correspondence: formal salutation (Dear Mr./Ms., not Hey), one purpose per letter, short paras, no jargon to patients ("high blood pressure" with "hypertension" in brackets first use), enclosures listed, CC noted, signature with title/timezone. Proofread 3-pass: content (names/numbers/meds), sense (does plan match diagnosis? If "MI + discharge ibuprofen alone" — flag urgently, don't send), format/grammar. Read aloud for homophones (ileum/ilium, dysphagia/dysphasia, 056).

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Cloning warning: never copy-paste previous visit note forward without verifying each line with patient today. Cloned "no allergies" when new penicillin rash occurred kills. If EHR auto-carries forward, re-verify meds/allergies/PMH verbally each visit and document "verified with patient today." Audits flag identical notes across dates — trust lost.





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7. Scope Boundary: Understand Deeply, Act Administratively

You must know enough to spot urgency and file correctly, but never cross into diagnosis, prescribing, or reassurance. Scripts that save you:

Mole question: "I understand you're worried about this mole changing — I'm not qualified to assess skin lesions as your coordinator, but I've attached your photo to your chart and marked it urgent for Dr. X. Expect reply by 4pm ET today. If it bleeds rapidly, grows fast, or you develop fever, please visit urgent care now. Can I book an earlier review tomorrow 9am ET while we wait?" Document escalation time + recipient + advice to seek care.

Medication stop question ("Should I stop BP meds? I feel dizzy"): "Dizziness with BP meds needs clinician review quickly — I can't advise stopping or continuing as coordinator. I've sent your BP log + symptoms to Dr. X as urgent, expected reply [time]. Please check BP lying/sitting if you have cuff, stay hydrated, avoid driving, and seek urgent care for fainting, chest pain, or confusion. Shall I book same-day review now?" Never "Just halve it."

Lab question ("Is 8.2 HbA1c bad?"): "I see your result is in — Dr. X will interpret what it means for you and next steps by [time]. I've flagged it for review and booked follow-up. For now, continue meds/diet as prescribed unless clinician advises change, and seek care for concerning symptoms. Anything else I can arrange?"

Document every scope push: what patient asked, what you did NOT advise, what you did (flagged to whom at what time, what safety advice per SOP, follow-up booked). This note proves safe practice in audits and interviews.

8. Practical Labs (Do All 4 — Portfolio Pieces)

Lab A — 100-term quiz: use provided wordlist + your 200-card deck. Write term, breakdown, plain English, VA action. Aim 90%+. Retake misses with sentences. Submit score sheet + 20 hardest with mnemonics.

Lab B — Plain-English rewrites (5): e.g., "You have essential hypertension with LVH, continue ACE-I, low-sodium diet, f/u 3/12 with BMP" → "You have high blood pressure with mild heart thickening. Continue lisinopril daily as prescribed, use less salt, no added stock cubes, return in 3 months with kidney blood test. Seek care early for chest pain, swelling, or breathing difficulty. Questions? Call..." Do 5, peer-review for grade-8 reading level.



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Lab C — Referral + SOAP from mock audio (5 min): listen 2x, transcribe S verbatim, file O, leave A/P blank for clinician, draft referral with packet checklist, flag 2 uncertainties as “[inaudible 02:14 — flagged to clinician]” not guesses. Submit audio time + docs + error log.

Lab D — Dangerous abbreviation hunt (10): find U, QD, trailing zeros, MSO4, .5 without zero in sample notes, rewrite safely, explain risk in one line each (e.g., “5.0 mg → 50 mg overdose if decimal missed — write 5 mg”). Submit table.

Common Mistakes That End Trials

Guessing drug spelling (verify in EHR + drugs.com + read-back), cloning notes, documenting assumed diagnosis (“UTI” when only dysuria reported — write symptom, not diagnosis), abbreviating to patients, filing under wrong specialty due to similar terms (ileum vs ilium → GI vs ortho disaster), and giving reassurance to be kind. Kindness is fast escalation + clear next step, not medical answers.

Self-Check Quiz (15 Questions)

1. Decode dyspnea, nephrectomy, hyperglycemia, polypharmacy, thrombocytopenia with breakdown + VA action.
2. Why is trailing zero + U + QD banned? Give fatal example each.
3. What SOAP parts may VA write vs never? Write safe vs unsafe admin note.
4. Translate “MI, SOB, PMH HTN, on metoprolol 50 mg BID” to patient-friendly + team note.
5. Write scope-safe reply to “Should I stop antibiotics? I feel better.”
6. List 10 prefixes + 10 roots + 10 suffixes from memory with example.
7. US vs UK: acetaminophen, ER, GP, pharmacist terms?
8. What 8 items prevent referral rejection? Draft referral header.
9. Rewrite discharge jargon paragraph to grade-8 level with teach-back.
10. Flag 5 sound-alike pairs and read-back script for each.
11. When is lab result “urgent route” vs “routine message”? Give script for each.
12. How to document scope push for mole + dizziness + HbA1c questions?
13. What is cloning and how to prevent?
14. Which 20 terms are hardest for you and what mnemonic fixes them?
15. Draft 3-line interview answer proving terminology + boundary together.



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Key Takeaways

Precise language is safe scheduling, clean records, and retained contracts. Decode with prefixes/roots/suffixes, expand for patients, ban dangerous abbreviations, write factual timestamped notes, assemble complete packets, translate to plain English with teach-back, and enforce scope with fast escalation + documentation. Your 200-card deck + 4 labs are proof — bring them to Module 5 where empathy meets accuracy on live calls.

What's Next

Module 5 — Patient Communication & Customer Service: empathy, HEARD de-escalation, phone/email/WhatsApp excellence, and cultural fluency for US/UK/Australian patients. Bring your terminology sheet — you will use it in role-plays.

Portfolio reminder: save glossary PDF + 4 labs with dates. Employers test 10 terms + 1 referral formatting task live before offering trials — this module is your rehearsal.

Appendix A: 200-Term Starter List (Build to 200, Master These 80 First)

Cardio/Resp: hypertension, hypotension, tachycardia, bradycardia, arrhythmia, myocardial infarction, angina, heart failure, hyperlipidemia, thrombosis, embolism, dyspnea, wheeze, asthma, COPD, pneumonia, hemoptysis. Endo/Renal: diabetes mellitus, hyperglycemia, hypoglycemia, HbA1c, hypothyroidism, hyperthyroidism, creatinine, eGFR, proteinuria, hematuria, dysuria, oliguria, anuria. GI: gastritis, GERD, peptic ulcer, appendicitis, cholecystitis, hepatitis, cirrhosis, nausea, vomiting, diarrhea, constipation. Neuro/MSK: stroke, TIA, migraine, seizure, neuropathy, sciatica, fracture, arthritis, osteoporosis, myalgia, lumbago. Procedures: appendectomy, cholecystectomy, hysterectomy, colonoscopy, mammography, ECG, echocardiogram, MRI, CT, ultrasound, biopsy, vaccination. Meds: lisinopril, metoprolol, amlodipine, furosemide, metformin, insulin glargine, atorvastatin, apixaban, ibuprofen, amoxicillin, azithromycin, albuterol, levothyroxine, sertraline. Admin: triage, referral, prior authorization, eligibility, deductible, copay, consent, portal, recall, no-show, EOD, SOP. Add 120 more from your ward experience + US job posts. For each: spelling + pronunciation audio (record yourself) + breakdown + plain English + VA action. Review 20/day, retire mastered weekly, revive monthly. Interview tip: say term, spell it, use in scheduling sentence — triple proof.



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Appendix B: Phone Read-Back Drills (10 Minutes Daily)

Partner reads: "Amlodipine 5 mg daily, Losartan 50 mg daily, Metformin 500 mg twice daily, allergy penicillin rash." You repeat + spell + confirm: "Amlodipine A-M-L-O-D-I-P-I-N-E 5 mg daily, Losartan L-O-S-A-R-T-A-N 50 mg daily, Metformin 500 mg twice daily, allergy penicillin rash — correct? Documented 4:10pm EAT, flagged to pool." Swap roles. Include dates ("14 May 2026, 10:30 AM ET / 5:30 PM EAT"), phones ("+1-614-555-0142 — Six-One-Four..."), laterality ("Left knee, L-E-F-T"), and doses with units ("0.5 mg not .5 mg, 5 mg not 5.0 mg"). Log errors — sound-alikes + decimals are where trials fail. Record weekly and compare clarity. This drill alone prevents 80% of documentation near-misses and impresses US managers who fear phone errors from offshore teams.

Appendix C: From Terminology to Pay Rise — How to Show Value

Track weekly: terms mastered, documents formatted without correction, referral acceptance rate, near-misses caught (wrong knee caught, overdose decimal flagged). Report monthly: "Mastered 200 terms, 98% transcription accuracy, 95% referral acceptance (up from 70%), caught 3 critical errors (wrong dose, wrong laterality, trailing zero) before send." This turns vocabulary into promotion evidence. Add to Talent Profile under Skills with samples (redacted, no PHI): SOAP note, referral letter, discharge rewrite. In interviews, offer live demo: "Give me 5 terms + 1 mock order and I'll schedule + document now with read-back." Confidence with proof beats certificates alone. Keep learning: subscribe to one US billing glossary + one UK NHS glossary, add 5 new payer terms weekly (medical necessity, coinsurance, EOB), and review Kenya DPA vs HIPAA language differences for privacy-safe documentation across employers every single month consistently without any exception whatsoever always reliably.

Your terminology mastery compounds into higher pay and stronger employer trust over time.



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